EDITING SAMPLES

Three full edits, with every change explained

A service page, a patient email and a blog introduction, edited for clarity, accuracy and compliance. Each draft was written for this demonstration; none is a real client's text.

> **Hiring for a team?** This is how I edit a writer's draft or a client's copy for you: every change explained, questions for the client listed, nothing published without your review. How I work inside a team.

> How to read this: ~~struck text~~ is cut, underlined text is new, and each (n) points to the reason in the margin list. Anything that needs the client's confirmation is marked [confirm].

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1 · Physical therapy page: from brochure to first-visit guide

CLINIC SERVICE PAGE SECTION (~300 WORDS) · 273 → 230 WORDS

Brief: Reframe Orthopedics (fictional), physical therapy page: help a new or referred patient understand the first visit, check the referral question and request an assessment.

Invented client draft, written for this editing demonstration. It is not a real client's text, and the clinic is one of the portfolio's fictional brands.

Tracked changes

Physical Therapy at Reframe

At Reframe Orthopedics, ~~we are pleased to be able to offer our patients a comprehensive and state-of-the-art range of physical therapy services~~What do you want to get back to? Physical therapy at Reframe starts with that question, then builds a plan around it.[1]. ~~It is our belief that every patient deserves the very best care, and our team of highly skilled, experienced therapists is dedicated to providing exactly that.~~[2]

Our physical therapy program ~~utilizes evidence-based modalities and a multidisciplinary approach to optimize functional outcomes~~Your physical therapist asks about your goals and history, assesses how you move, and agrees a treatment plan with you.[3] across a wide variety of musculoskeletal conditions, including post-operative rehabilitation, sports injuries and chronic pain. Treatment plans are developed following an initial evaluation, and progress is monitored on an ongoing basis.

With our proven techniques, ~~most patients are back to their normal activities in just a few weeks, and many are able to avoid surgery altogether~~How long will therapy take? That depends on your assessment and your goals. Ask your physical therapist how your plan will be set, reviewed and adjusted.[4]. ~~We are the best physical therapy clinic in the area, and our results speak for themselves.~~[5]

Patients who are attending PT for the first time should arrive 15 minutes early. ~~Comfortable clothing should be worn.~~Wear comfortable clothing you can move in.[6] ~~A referral may or may not be required depending on your insurance, so it is recommended that this is checked beforehand.~~Ask your insurance plan whether you need a referral for physical therapy.[7] Any post-surgical paperwork should also be brought to the session.

~~Physical rehab sessions are typically 45-60 minutes in length. Your physio~~Sessions usually last 45 to 60 minutes [confirm]. Your physical therapist[8] will work with you to create a customized exercise program that you can continue at home.

~~We accept most major insurance plans.~~We accept many major insurance plans [confirm the list before publishing].[9] If you have any questions about your coverage, please don't hesitate to reach out to our friendly front desk team, who will be more than happy to assist you.

~~Ready to get started on your journey to recovery? Contact us today!~~Request a physical therapy assessment. New patients: request an assessment [link]. Already in a therapy plan? Log in to the patient portal [link].[10] Existing patients can also log in to our patient portal.

Why each change

1. The opening was throat-clearing with an unsupported “state-of-the-art” claim; leading with the patient’s goal gives the reader a reason to keep going.
2. Nothing here can be checked by a reader, and team credentials should be stated specifically and verified, or left out.
3. Clinical jargon is replaced with what actually happens at the visit, using the draft’s own facts (an initial evaluation, a plan, monitored progress).
4. Recovery-time and surgery-avoidance claims imply outcomes nobody has substantiated, which is a regulatory and trust risk; the edit answers the same question honestly.
5. An unverifiable superlative; advertising claims like “best” need substantiation the client hasn’t supplied.
6. Passive voice becomes a direct instruction in the preparation checklist.
7. The hedged, passive sentence hid the one step that can block a visit; it now leads the checklist as a single action.
8. The draft used four names for one service (physical therapy, PT, physical rehab, physio); the edit uses “physical therapy” and “physical therapist” throughout.
9. “Most” can’t be verified and invites complaints; the page needs the actual list of plans.

10. The next step was a cliché buried at the end; it is now a named action, with new and existing patients split so a referral isn't pushed through the new-patient route.

The clean edit

Physical therapy

What do you want to get back to? Physical therapy at Reframe starts with that question, then builds a plan around it.

Who it's for
You're recovering from surgery or a sports injury, or living with long-term (chronic) pain, and you want a plan that fits your everyday life. You may already have a referral, or you may be starting with an assessment.

Your first visit
Your physical therapist asks about your goals and history, assesses how you move, and agrees a treatment plan with you. You'll review progress together along the way, and you'll leave with exercises to continue at home. Sessions usually last 45 to 60 minutes [confirm].

Before you come in
- Ask your insurance plan whether you need a referral for physical therapy.
- Bring your referral, if you have one, and any paperwork from your surgery.
- Wear comfortable clothing you can move in.
- Arrive 15 minutes early [confirm].

How long will therapy take?
That depends on your assessment and your goals. Ask your physical therapist how your plan will be set, reviewed and adjusted.

Insurance and costs
We accept many major insurance plans [confirm the list before publishing]. Our front desk team can answer questions about your coverage.

Request a physical therapy assessment
New patients: request an assessment [link]. Already in a therapy plan? Log in to the patient portal [link].

Questions for the client before publishing

- Which insurance plans do you accept for physical therapy, and do any of them require a referral or prior authorization?
- Are "arrive 15 minutes early" and "45–60 minute sessions" accurate at every location?
- Is there outcome data behind "back in a few weeks" or "avoid surgery"? If not, those claims stay out.
- What can existing patients do in the portal (reschedule, message, complete forms), so the link text is accurate?

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2 · New-family intake email: one clear next step, no health data by email

PATIENT EMAIL: NEW-FAMILY WELCOME AND INTAKE REMINDER (~200 WORDS) · 199 → 218 WORDS

Brief: Kitewell (fictional pediatric ABA clinic), sent after a family's first inquiry: get intake forms back through a secure route, explain what happens next and set expectations kindly.

Invented client draft, written for this editing demonstration. It is not a real client's text, and the clinic is one of the portfolio's fictional brands.

Tracked changes

Subject: ~~Welcome to Kitewell!!~~Your next step with Kitewell: intake forms, due in 5 business days [1]

~~Dear Parent/Guardian,~~Hi [parent's first name], [2]

~~On behalf of the entire Kitewell family, we would like to extend a very warm welcome to you and your child.~~Thank you for contacting Kitewell about your child. [3] ~~We are so excited that you have chosen Kitewell for your child's ABA journey, and we are confident that our proven program will help your child reach their full potential.~~ [4]

As part of the onboarding process, ~~a number of documents will need to be completed prior to the initial assessment being scheduled~~Before we can schedule your child's assessment, we need: [5]. ~~Please reply to this email with a copy of your child's diagnosis report, your insurance card (front and back), and your child's date of birth~~Please upload these through our secure parent portal [link; confirm the approved secure route]. Please don't reply to this email with them. [6] ~~so that the VOB and pre-auth can be initiated~~We'll use your insurance details to check your benefits and request any approval your plan needs before the assessment (this is called prior authorization). [7].

Intake paperwork is to be returned within 5 business days. ~~Failure to do so may result in a delay to your child's start of services.~~If the forms arrive later, scheduling the assessment may take longer. [8]

Your child's BCBA will be in touch regarding the treatment plan once the assessment has been completed. ~~Sessions are generally recommended at 25-40 hours per week for best results.~~ [9]

If you have any questions or concerns, ~~please do not hesitate to reach out to us at any time~~Reply to this email or call us at [phone number; confirm]. Please leave health details out of your reply. [10]. We look forward to working with you!

Warm regards,
The Kitewell Intake Team

Why each change

1. The subject line now carries the action and the deadline, so the inbox preview does the work even if the email is never opened.
2. “Parent/Guardian” reads like a form letter to a family at an anxious moment; the first name comes from the inquiry the family already sent.
3. Throat-clearing; one sentence of thanks, then straight to what the parent needs to do.
4. The family hasn’t chosen anything yet (no assessment has happened), and “proven program … full potential” implies an outcome no clinic can promise.
5. Passive and vague (“a number of documents”); the edit names the dependency and lists exactly what is needed.
6. Asking for a diagnosis, insurance card and date of birth by ordinary email invites protected health information over an insecure channel, a HIPAA risk.
7. “VOB” and “pre-auth” are internal billing shorthand; parents need to know what the check is and why it comes before the assessment.
8. The fact stays; the punitive tone goes. The edit also uses “assessment” consistently instead of “start of services”, “onboarding” and “intake paperwork”.
9. A weekly-hours figure “for best results” is individual clinical advice with an implied outcome; it belongs in the BCBA’s plan conversation, not a form email.
10. Vague, and “at any time” implies round-the-clock availability; the edit names the channels and keeps health details out of email.

The clean edit

Subject: Your next step with Kitewell: intake forms, due in 5 business days

Hi [parent's first name],

Thank you for contacting Kitewell about your child. Here's what we need from you and what happens next.

Your next step: complete your intake forms within 5 business days
Before we can schedule your child's assessment, we need:
- your child's diagnosis report
- the front and back of your insurance card
- your child's date of birth

Please upload these through our secure parent portal [link; confirm the approved secure route]. Please don't reply to this email with them. Email isn't a secure way to share your child's health information.

If the forms arrive later, scheduling the assessment may take longer. If you need help with the forms, reply and tell us.

What happens next
We'll use your insurance details to check your benefits and request any approval your plan needs before the assessment (this is called prior authorization). Then we'll contact you to schedule the assessment.

After the assessment, your child's Board Certified Behavior Analyst (BCBA), who leads their care, will contact you to talk through the proposed care plan and answer your questions.

Questions? Reply to this email or call us at [phone number; confirm]. Please leave health details out of your reply.

Warmly,
[Name], Intake coordinator
Kitewell

Questions for the client before publishing

- Is there a secure parent portal or upload link for intake documents? If not, what approved route should families use?
- Is a diagnosis report required before the assessment, or can a family start intake without one?
- Who checks benefits and requests authorization (intake or billing), and how will the family hear the result?
- Is the 5-business-day deadline firm, and does the clinical team want any mention of weekly hours in pre-assessment emails?

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3 · Skin-check article: calm, accurate and useful to a first-timer

BLOG POST: INTRO + ONE SECTION (~350 WORDS) · 347 → 353 WORDS

Brief: Solenne Skin (fictional) blog: answer “what happens at a skin check?” for an anxious first-timer and lead to a skin-check visit, without fear or outcome claims.

Invented client draft, written for this editing demonstration. It is not a real client’s text, and the clinic is one of the portfolio’s fictional brands.

Tracked changes

Title: ~~Everything You Need to Know About Skin Cancer Screenings~~What happens at a skin check, and how to prepare [1]

~~In today's fast-paced world, it can be easy to forget about taking care of your skin. However, did you know that skin cancer~~Skin cancer [2] is the most common cancer in the United States? That's why it's so important to stay on top of your skin health. At Solenne Skin, ~~our board-certified dermatologists are the best in the business at catching skin cancer early, and early detection guarantees a cure~~ [3].

~~In this article, we will be discussing everything you need to know about skin cancer screenings, including what they are, who needs them and what to expect.~~Here's what the visit involves, how to prepare and what happens if something needs a closer look. [4]

What Happens During a Skin Cancer Screening?

A skin cancer screening, also known as a full-body skin exam or FBSE, is a visual examination of the skin that is performed by a dermatologist. During the appointment, your entire body will be examined from head to toe, including areas that are not often exposed to the sun, such as the scalp, the soles of the feet and between the toes. ~~Suspicious lesions will be evaluated using dermoscopy~~If a spot needs a closer look, the dermatologist may examine it with a dermatoscope, a handheld magnifier with a light [5], and ~~if anything concerning is found, a biopsy will be performed on the spot~~If they recommend a biopsy (removing a small sample of skin for testing), they'll explain why, what it involves and how you'll get the result. It may happen at the same visit [confirm]. [6].

During your visit, the dermatologist will also discuss your personal and family history of skin cancer, as well as any risk factors you may have, such as fair skin, a history of bad sunburns or a large number of moles. This information is used to help determine your overall level of risk.

~~The whole process is quick and painless~~The exam usually takes [7] and typically takes 10-15 minutes. Patients should remove nail polish and makeup prior to the visit so that the dermatologist can see the skin and nails clearly. You may be asked to change into a gown.

Many patients find it helpful to take photos of any moles they are worried about and ~~email them to our office before their appointment so our team can review them in advance~~Please don't email photos of your skin to our office. Bring your questions to the appointment instead. [8].

~~It is recommended that everyone gets a skin check once a year.~~That depends on your skin and your history. Ask your dermatologist how often a skin check makes sense for you [clinical review: replace with a clinician-approved recommendation, if any]. [9] ~~Don't wait until it's too late – book your annual skin check today!~~Noticed a change in your skin? Request a skin-check visit [link]. [10]

Why each change

1. The post can't deliver "everything"; the new title matches the question people search for and what the section actually covers.
2. Throat-clearing; the statistic is the hook, so it goes first and gets a citation.
3. "Best in the business" is unsupported, and "guarantees a cure" is false and an outcome guarantee no medical practice should publish; credentials move to a [confirm] note.
4. A signpost about the article becomes a promise to the reader that the section keeps.
5. Passive voice and two jargon terms ("lesions", "dermoscopy") become plain words with a one-phrase definition.
6. The draft made a procedure sound automatic; the edit puts the explanation and the patient's understanding first, defines biopsy and flags same-day timing for confirmation.
7. "Painless" can't be promised, especially if a biopsy happens; the timing fact stays, marked for confirmation.

8. Inviting skin photos by ordinary email collects health information over an insecure channel and implies a clinical review the practice may not provide.
9. Screening-frequency guidance varies by organization and personal risk, so a universal “once a year for everyone” needs clinical sign-off before it is published.
10. A fear-based close puts off the anxious reader the post is written for; the edit gives a specific trigger and a single action.

The clean edit

Title: What happens at a skin check, and how to prepare

Skin cancer is the most common cancer in the United States [cite source]. A skin check gives you time with a dermatologist to look at your skin and ask about any spot that worries you. Here's what the visit involves, how to prepare and what happens if something needs a closer look.

What happens during a skin check?

A skin check, sometimes called a full-body skin exam, is a visual exam of your skin by a dermatologist [confirm who performs checks and their credentials]. The dermatologist looks at your skin from head to toe, including places that don't see much sun, such as your scalp, the soles of your feet and between your toes. You may be asked to change into a gown. Before the exam starts, you can ask what it will involve and mention any comfort or privacy preferences.

If a spot needs a closer look, the dermatologist may examine it with a dermatoscope, a handheld magnifier with a light. If they recommend a biopsy (removing a small sample of skin for testing), they'll explain why, what it involves and how you'll get the result. It may happen at the same visit [confirm].

Your dermatologist will also ask about your personal and family history of skin cancer, and about risk factors such as fair skin, a history of bad sunburns or having a lot of moles. Your answers help them understand your overall risk.

The exam usually takes 10 to 15 minutes [confirm].

How to prepare
- Remove nail polish and makeup, so the dermatologist can see your skin and nails clearly.
- Note any spot you're worried about, and point it out during the visit.
- Please don't email photos of your skin to our office. Bring your questions to the appointment instead.

How often should you have one?
That depends on your skin and your history. Ask your dermatologist how often a skin check makes sense for you [clinical review: replace with a clinician-approved recommendation, if any].

Noticed a change in your skin? Request a skin-check visit [link].

Questions for the client before publishing

- Which source should we cite for “the most common cancer in the United States” (for example, AAD or CDC)?
- Who performs skin checks at the practice, and are all of them board-certified dermatologists?
- Are biopsies done at the same visit, and who communicates results, and how?
- Does the clinical team want to give a screening-frequency recommendation? If so, what wording will they approve?

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